



Physical health care: mental health consumers

1. Background

1.1. About this document

People living with mental illness especially severe mental illness are more likely to experience poorer physical health and tend to die earlier than the general population.¹ Almost 80% of premature deaths in people with mental illness are due to physical health conditions² such as cardiovascular disease, cancer, respiratory illnesses and diabetes.³

South Metropolitan Health Service (SMHS) is committed to supporting mental health consumers to achieve positive physical and mental health outcomes by addressing physical health risks, focusing on prevention and management of co-occurring physical health conditions, supporting the adoption and maintenance of health promoting behaviours. This guideline informs practice and enables SMHS mental health clinicians to provide care compliant with the Chief Psychiatrists Clinical Care Standard: Physical Health care of mental health consumers.

This guideline does not address the specific needs of people who present as medically unstable or with acute deterioration e.g. people with eating disorders can be at high risk of medical complications and may present in more acute and urgent stages of physical ill health.⁴

Refer to:

- [FSH Code Blue](#)
- [FSH CH Code Blue](#)
- [FH Code Blue](#)
- [RkPG Code Blue](#)

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1.3 Key definitions

Term	Definition
Cardiometabolic risk	Genetic, environmental, personal and medical factors and illness-related behaviours, associated with the development of metabolic and cardiovascular disease.
Carer	A carer provides unpaid, ongoing care to support family members and friends who have a disability, mental illness, chronic condition, terminal illness, an alcohol or drug issue or who are frail aged. Carers Recognition Act 2004 s. 5.
Eating Disorder	Eating disorders are serious, complex, and potentially life-threatening mental illnesses. They are characterised by disturbances in behaviours, thoughts and feelings towards body weight and shape, and/or food and eating.
Malnutrition	Refers to deficiencies or excesses in nutrient intake (e.g. undernutrition and overweight and obesity), imbalance of essential nutrients or impaired nutrient utilisation, and the resulting diet-related non communicable diseases. Undernutrition manifests as wasting, stunting, underweight and micronutrient deficiencies.
Metabolic syndrome	A cluster of cardiovascular risk factors including insulin resistance, hypertension, central obesity and dyslipidaemia, which result in significantly increased risk of cardiovascular disease, type 2 diabetes and mortality. Metabolic syndrome is related to several factors including but not limited to medications, lifestyles factors, psychosocial factors and comorbid physical illness.

Multidisciplinary Team (MDT)	A team of mental health staff usually comprised from a range of disciplines including psychiatry, General Practitioners, psychology, nursing, occupational therapy, social work, dietetics, exercise physiologists, physiotherapists, pharmacy, peer workers, welfare officers, Aboriginal Mental Health Liaison and other relevant groups who collaborate in the assessment, planning and delivery of care.
Negative medication effects	Includes adverse drug reactions, side effects and drug interactions.
Personal support person (PSP)	Can be any of the following people who are supporting someone who is experiencing mental illness – close family member, carer, their nominated person; the parent or guardian of a child or any enduring guardian of an adult Mental Health Act (MHA) 2014 s.7. Some consumers may use the term kin to refer to a family member.
Physical health	Relates to the physical body and includes impacts on bodily functions and structures, and a person's ability to engage in a range of activities and participate in community life.
Polypharmacy	Psychiatric polypharmacy refers to combination therapy with two or more psychotropic medicines. It has been associated with an increased side effect burden, high dose prescribing, increased hospitalisation and length of stay rates and increased mortality.
Recovery oriented practice	An approach to care delivery that emphasises the importance of understanding that each person is different and should be supported to make their own choices, listened to and treated with dignity and respect. Each person is the expert of their own life and support should assist them to achieve their hopes, goals and aspirations. Recovery will mean different things to different people.
Self-management	Recognises the ability of individuals to manage their health and wellbeing, with support. Supporting people to self-manage can result in significant physical health gains, such as improved symptom management.
Social determinants of health	Are the conditions in which people are born, grow, work, live and age, and the wider forces that shape the conditions of daily life. Most of a person's health is determined by these non-medical root causes of ill health, which include quality education, access to nutritious food, and decent housing and working conditions.
Trauma informed care	A strengths-based service delivery approach 'that is grounded in an understanding of and responsiveness to the impact of trauma, that emphasises physical, psychological and emotional safety for both providers and survivors, and that creates opportunities for survivors to rebuild a sense of control and empowerment'. The core trauma informed principles are safety, trustworthiness, choice, collaboration, empowerment and respect for diversity.

2. Guidelines

2.1. Principles of practice

Many factors contribute to the poorer physical health outcomes experienced by mental health consumers, including:

- Higher rates of substance use
- Higher prevalence of unhealthy lifestyles
- Lower access to and participation in health promoting behaviours (e.g., physical activity, sleep, healthy diet) Negative medication effects particularly metabolic side effects of antipsychotics
- Inequalities of access to and use of health care²
- Social determinants of health.

SMHS MH will provide physical health screening, assessment and care in a respectful, non-judgemental manner that is person centred, recovery oriented, trauma informed and acknowledges and understands cultural differences and needs.

Physical health screening and intervention is an essential component of care across inpatient and community mental health settings. It aims to identify people at risk of negative physical health outcomes, identify if further assessment is warranted, and support the development of a comprehensive treatment plan. Physical health screening should include behavioural questions, consideration of health literacy and accessibility challenges, a physical health assessment and pathways for intervention and education as needed.⁵

Aboriginal people are estimated to have ten years lower life expectancy than other Australians, with an even greater gap for those with mental illness.⁶ Exposure to chronic stress including intergenerational trauma throughout life may contribute to a number of metabolic, cardiovascular and mental disorders that shorten life expectancy in Aboriginal people.⁶

Where appropriate and possible clinicians must consult with Aboriginal Mental Health Liaison / Aboriginal Health Liaison, specialist Aboriginal health advisory groups and/or community to ensure the approach to providing physical health care for Aboriginal consumers recognises the physical wellbeing of an Aboriginal person is holistic, encompassing physical, social, emotional, cultural and spiritual aspects. The screening process should also include a cultural assessment which can be facilitated through the Aboriginal Mental Health Liaison/ Aboriginal Health Liaison or community.

Interpreter services will be used to facilitate treatment planning processes for consumers from culturally and linguistically diverse backgrounds.

Peer worker support for physical healthcare and peer-facilitated interventions will be provided to improve physical health outcomes.³

2.2. Carers / Personal Support Persons

Where the consumer gives verbal permission, the carer / PSP will be informed regarding the consumer's physical health status and involved in physical care and treatment planning where appropriate.

The consumer's carer / PSP or Aboriginal Mental Health Liaison / Aboriginal Health Liaison may be engaged to support physical health screening and assessment where appropriate.

Carers / PSP should be informed about the relative risks of the consumer developing a physical illness, and screening protocols for diabetes, cardiovascular disease, obesity, respiratory disease, osteoporosis, dental and oral health.⁷

Refer to:

- [SMHS Mental Health rights of carers and other personal support persons](#)

3. Screening and assessment

SMHS MH services will provide an appropriate room and equipment for the physical health examination of a consumer for both the inpatient and community settings. Refer to [Appendix A](#) Checklist for equipment and support for physical examination.⁷

3.1. Inpatient

As per section 241 of the [MHA 2014](#) initial physical health screening will occur as soon as practicable or within 12 hours of admission to an acute inpatient service. Where a consumer is admitted voluntarily the consumers consent is required to conduct the examination, they cannot be compelled to have a physical examination. However, if it appears their refusal is a symptom of a mental illness it may be appropriate to review if the consumer remains as a voluntary inpatient.

If a consumer is involuntary, consent is not required however it is good practice to seek informed consent. Where a person is too distressed, unable to participate, or refuses assessment, the assessment may be postponed and completed at the earliest opportunity with the consumer's cooperation. This must be documented in the consumer's medical record. Refer to [Appendix B](#) Minimum standards for physical health screening on the initial admission in the inpatient setting.^{4,7} Screening required on subsequent inpatient admissions will be determined based on the individual consumer's presentation and clinical judgement.

Prior to the physical health screen staff should consider the consumers history, gender and cultural background to determine the appropriate clinician and / or requirement for chaperone to ensure the sexual and cultural safety of the consumer. The clinician will explain all procedures to the consumer before conducting them⁵ and request consent from the consumer, documenting consent in the consumer's medical record. Hand hygiene will be performed where indicated.

Refer to:

- [SMHS Chaperone policy.](#)
- [SMHS Hand hygiene and skin integrity](#)

On admission to a mental health inpatient ward routine screening will also include:

- skin integrity bundle
- venous thromboembolism (VTE) risk
- bowel assessment as part of nursing admission.

The results of the physical examination will be documented in the consumer's medical record on the Mental Health Physical Examination Form. Additional screening and/or investigations will be performed as clinically indicated depending on results from initial the physical examination and investigations and the patients past medical history

The consumer's General Practitioner (GP) (if they have one) should be contacted to provide a summary of past and current medical issues and medications.⁶

Mental health inpatient admissions should be used as an opportunity to improve consumer involvement and access to preventative health care.⁸

3.2. Community

Consumers with severe mental illness are more than twice as likely to be smokers or risky drinkers and significantly more likely to have hypertension or dyslipidaemia or be identified as being obese.⁹ While people with mental illness have a high morbidity burden, many of the risk factors and conditions identified are modifiable or manageable.⁹

Physical health screening will initially occur within one month of admission to community care. Refer to [Appendix B](#) Minimum standards for initial physical health screening in the community setting.^{4,7} The physical examination will be documented in the consumers medical record using the Mental Health Physical Examination Form where possible.

Additional screening tools where indicated may include:

- [Oral health checklist](#)
- [Abnormal Involuntary Movement Scales \(AIMS\)](#)

On entry to the mental health service, a consumer's General Practitioner (GP) (if they have one) should be contracted to provide a summary of past and current medical issues and medications.⁶

Follow up physical health assessments in the community will occur:

- as indicated by consumers medical history, concerns raised by consumer, results of screening or the presence of new signs and symptoms relating to cardiometabolic risks
- at any change of psychotropic medication
- as required for consumers prescribed clozapine
- at a minimum annually in community care
- for consumers 65 years or older, or who have as significant physical illness or disability, this should be no less frequently than six monthly
 - In older age, physical and mental health care are particularly closely linked.⁶ Mental health, General Practice and health services should support a coordinated approach to physical health in older people

Refer to:

- [Appendix C](#) Follow up assessment checklist⁷
- [Appendix D](#) Metabolic Syndrome Early Intervention provides recommended interventions and target parameters to guide appropriate referral and monitoring in the community.
- [Guidelines for the Safe and Quality Use of Clozapine Therapy in the WA Health System](#)

At three-monthly clinical reviews in the community:

- review the consumer's blood pressure, weight and waist circumference
- review recent results and referral outcomes
- monitor side effects for consumers on antipsychotic medications including but not limited to dizziness/light headedness, dry mouth, abnormal vision, weight gain, constipation or involuntary movement or involuntary muscle contractions.⁵
- review the date of the most recent physical health examination

Community mental health clinicians can collaborate with the consumers GP to complete physical health screening and are responsible for ensuring these results are made available in the consumer's medical record.

4. Comprehensive Treatment Plan

4.1. Inpatient

Screening information will be used to formulate a cardiometabolic risk profile on admission and prior to starting or changing psychotropic medications.⁷ The psychiatrist should inform and discuss cardiometabolic risks with the consumer and involve them in treatment planning.⁷

Following assessment consumers will be commenced on evidence based therapeutic interventions appropriate to their bio-psychosocial needs.¹⁰ For consumers with eating disorders refer to specific Models of Care and policies to guide care:

- [RkPG Eating Disorders: Multidisciplinary team review process for persons presenting with Guideline](#)
- [FSFH MH Youth Unit Eating Disorder with Co-morbidity Treatment Model of Care](#)
- [FSH – Cockburn Health Eating Disorders Unit Model of Care](#)

The comprehensive treatment plan will:

- identify and address any acute or chronic physical health concerns
- collaborate with the consumers GP for follow up care
- seek advice from physical health colleagues⁸ if indicated
- early referral to specialists for intervention⁷
- refer to appropriate allied health e.g. exercise physiology, physiotherapist, dietitian

The agreed plan for the consumer's ongoing physical health care will be documented in their Treatment Support and Discharge Plan (TSDP) and medical records. TSDPs will be reviewed regularly and as required and followed up.⁶

Staff will communicate information to the consumers GP via the discharge summary on the consumer's mental and physical health and required follow up.

Refer to:

- [Chief Psychiatrists Guideline The preparation, review and revision of treatment, support and discharge plans](#)

- [SMHS Discharge Summary](#)

4.2. Community

Following assessment consumers will be promptly commenced on evidence based therapeutic interventions appropriate to their bio-psychosocial needs.¹⁰ For consumers with eating disorders refer to the:

- [Kara Maar SMHS Specialist Community Eating Disorder Service Model of Care.](#)

Lifestyle based interventions are recommended as a foundational component of mental health care for adults with major depressive disorder including the use of physical activity and exercise, relaxation techniques, work directed interventions, sleep and mindfulness-based therapies.¹¹ The community MDT will support strategies to minimise negative medication effects and promote health.

Screening information will be used to formulate a cardiometabolic risk profile at initial assessment and prior to starting or changing psychotropic medications.⁷ The psychiatrist should inform and discuss cardiometabolic risks with the consumer and involve them in treatment planning.⁷

The comprehensive treatment plan will:

- reflect consumer preferences and available resources¹¹
- incorporate behaviour change techniques¹¹ and promote self-management
- collaborate with the consumers GP to complete physical health screening
- identify and collaborate with existing services the consumer may be receiving for chronic disease conditions and/or disability
- seek advice from physical health colleagues⁸
- early referral to specialists for intervention⁷
- refer to appropriate allied health e.g. exercise physiology, physiotherapist, dietitian
- refer to appropriate Aboriginal Community Controlled Health Organisations (ACCHO) or Aboriginal Community Controlled Organisations (ACCO).

In older age, physical and mental health care are particularly closely linked.⁶ Mental health, General Practice and health services should support a coordinated approach to physical health in older people.

The agreed plan for the consumer's ongoing physical health care will be documented in their TSDP and medical records. TSDPs will be regularly reviewed (at a minimum every three months) and followed up.⁶ The consumer's care coordinator will promote engagement and choice to support the consumer to be actively involved in their care. Consider the need to support community mental health consumers to attend scheduled appointments by organising transport.

Wherever possible the consumer will be linked with appropriate primary care and community-based service providers with an agreed plan to support their physical health and wellbeing.

GPs can engage and support consumers to participate in screening and monitor investigations and health checks focused on regular assessment and management of cardiovascular risk, blood pressure, cholesterol levels and smoking cessation. GPs also promote participation in regular cancer screening programs³ and the regular health screening recommended by age and gender including the reproductive life cycle for women.

Staff will ensure evidence of communication with the consumers GP (e.g. letters, progress notes, details of phone conversations, Care Transfer Summary) providing information on the consumer's mental and physical health and agreed treatment plan is made available in the consumer's medical record.

5. Promotion and prevention

5.1. Inpatient

Engage with Aboriginal Mental Health Liaison / Aboriginal Health Liaison to support effective and appropriate communication with Aboriginal consumers and carers / PSP or utilise the interpreter service as required.

The inpatient MDT will support strategies to minimise negative medication effects and promote health. In the inpatient setting this may include:

- access to purposeful and predictable therapeutic program⁴
- opportunities to maintain and increase mobility, physical activity and function⁴ though access to exercise physiology, physiotherapy and onsite gyms
- provision of appealing and healthy, culturally appropriate food choices to maintain and improve nutrition⁴
- nicotine replacement therapy and support
- education about oral hygiene⁴
- enhancing physical health literacy⁴

Practitioners prescribing medications will comprehensively discuss with the consumer specific treatment goals, the risks (including interactions) and benefits and a timescale for response.¹⁰ Consumers will be offered written information, and it will be documented if the consumer declines, using:

- [Choice and Medicine® leaflets](#) from the WA Health Department of Health for psychotropic medications
- Consumer Medicine Information approved by the Medicines and Technology Unit (MTU) or the Therapeutic Goods Administration for non-psychotropic medications.

5.2. Community

Consumers should be informed about the relative risks of developing a physical illness, and screening protocols for physical health conditions for example diabetes, cardiovascular disease, obesity, respiratory disease, osteoporosis, dental and oral health.⁶

Access to health promotion, screening and preventative activities will be made available to the consumer including physical activity, relaxation, smoking cessation, healthy eating, weight management strategies and sexual health. Written information will be made readily available to consumers and carers on general physical health promotion strategies.

Practitioners prescribing medications will identify those consumers eligible for an exemption from prescription co-payment and follow site-based protocols. This includes:

- Consumers on a Community Treatment Order
- Consumers prescribed weekly clozapine
- Closing the Gap consumers
- Consumers with severe financial hardship where the inability to pay is likely to lead to inability to access prescribed treatment.

All health promotion information and advice provided to the consumer will be documented in the consumer's medical record.

6. Compliance Monitoring

Service Directors will monitor compliance with this guideline via one or more of the following:

- WECAN compliance rate for physical examination within 12 hours of admission to acute inpatient services
- Annual Treatment Support and Discharge Plan audit evaluating documentation of the consumers physical health care treatment plan
- Regular completion of HoNOS scores
- Routine clinical incident monitoring related to missed or delayed physical health interventions

7. Legislation

- [Carers Recognition Act 2004](#)
- [Mental Health Act 2014](#)

8. Related Documents

8.1. Department of Health policies

- [MP 0155/21 Statewide Standardised clinical documentation for mental health services policy](#)
- [Statewide Standardised clinical documentation for mental health services procedure](#)
- [Triage to discharge Mental Health Framework for Statewide Standardised Clinical Documentation](#)
- [Guidelines for the Safe and Quality Use of Clozapine Therapy in the WA Health System](#)
- [MP 0122/19 Clinical Incident Management Policy](#)

8.2. SMHS policies

- [Discharge Summary SMHS: 7](#)
- [Care Coordination in Mental Health SMHS: 13](#)
- [Hand hygiene and skin integrity](#)
- [Chaperone SMHS: 169](#)

8.3. Other documents

- [Chief Psychiatrists Clinical Care Standard: Physical Health care of mental health consumers](#)
- [Chief Psychiatrists Clinical Care Standard: Transfer of Care](#)
- [Australian Government Department of Health, Disability and Ageing Gayaa Dhuwi \(Proud Spirit\) Declaration Framework and Implementation Plan](#)

8.4. References

1. Australian Government. Australian Institute of Health and Welfare. (2025) Physical health of people with mental illness. <https://www.aihw.gov.au/reports/mental-health/physical-health-of-people-with-mental-illness>. Last updated 20 May 2025.
2. Lawrence D, Hancock KJ and Kisely S (2013) The gap in life expectancy from preventable physical illness in psychiatric patients in Western Australia: Retrospective analysis of population-based registers. *BMJ* 346: f2539.
3. Calder R. V., Dunbar J. A., & de Courten M. P. (2022). The Being Equally Well national policy roadmap: providing better physical health care and supporting longer lives for people living with serious mental illness. *Med J Aust*, 217 Suppl 7(Suppl 7), pp. S3-s6.
4. New South Wales. Ministry of Health (2021). Physical health care for people living with mental health issues. Ministry of Health.
5. Finlay A. Evidence Summary. Severe mental illness: Physical health screening and assessment. The JBI EBP Database. 2024; JBI-ES-5298-1.
6. National Mental Health Commission. Equally Well Consensus Statement: Improving the physical health and wellbeing of people living with mental illness in Australia. Sydney: NMHC, 2016.
7. Lambert TJ, Reavley NJ, Jorm AF, Oakley Browne MA. Royal Australian and New Zealand College of Psychiatrists expert consensus statement for the treatment, management and monitoring of the physical health of people with an enduring psychotic illness. *Aust N Z J Psychiatry*. 2017;51(4):322-337
8. Koomson D., Shotton H., Docherty M., & Srivastava V. (2024). A picture of health? The quality of physical healthcare provided to adult patients admitted to a mental health inpatient setting. *British Journal of Hospital Medicine*, 85(3), pp. 1-4.
9. Belcher J, Myton R, Yoo, J et al. Exploring the physical health of patients with severe or long-term mental illness using routinely collected general practice data from

MedicineInsight. Australian Journal of General Practice. 2021;50(12). doi: 10.31128/AJGP-08-20-5563

10. Royal College of Psychiatrists. College Centre for Quality Improvement. Standards for Inpatient Mental Health Services. Fourth Edition. 2022.
11. Marx W, Manger S, Blencowe m ET AL. (2023) Clinical Guidelines for the use of lifestyle based mental health care in major depressive disorder: World Federation of Societies for Biological Psychiatry (WFSBP) and Australasian Society of Lifestyle Medicine (ASLM) taskforce, The World Journal of Biological Psychiatry, 24:5, 333-386, DOI: 10.1080/15622975.2022.2112074

8.5. Version Control

Version	Published date	Effective from	Review date	Effective to*	Amendment (s)
GP V1	2 April 2026	27 March 2026	27 March 2029	March 2029	Original version

8.6. Approval

Approval by	Executive Director Mental Health
Approval date	20 March 2026

9. Appendix A: Checklist for equipment and support for physical examination

Equipment
☐ Hand washing facilities ☐ Disposable / sterile gloves ☐ Privacy gowns ☐ Tape measures (non-stretch) ☐ Ophthalmoscope ☐ Pen torch ☐ Patella hammer ☐ Digital scales ☐ Height measure (calibrated rigid stadiometer) ☐ Glucometer and accessories ☐ Stethoscope ☐ Sphygmomanometer ☐ Electrocardiogram machine ☐ Urinalysis kit (dipstick for microscopic haematuria, proteinuria and infection and ketones) ☐ Access to pathology ordering and online results ☐ Appropriate biohazard disposal units
Other
☐ Suitable room ☐ Chaperone as required

Adapted from RANZCP expert consensus statement for the treatment, management and monitoring of the physical health of people with an enduring psychotic illness 2017.

Appendix B: Physical Health Screening Checklist

Initial assessment checklist				
		Inpatient	Community	Notes for specific populations and settings
Cardiometabolic screening	Lifestyle factors			
	Smoking	✓	✓	Within 12 hours in acute services Opportunistic screening, assess stage of change and offer cessation assistance including nicotine replacement therapy
	Diet / malnutrition / hydration	✓	✓	Within 1 month of admission to community care Review as part of metabolic monitoring
	Physical activity	✓	✓	
	Screen time and technology use	✓	✓	
	Sexual health	✓	✓	
	Weight / abdominal obesity			
	Weight and height	✓	✓	Within 12 hours in acute services, then weekly. Within 1 month of admission to community care
	Waist circumference (WC)	✓	✓	Once stable, review as part of metabolic monitoring Check height, weight, BMI, WC and check trends
	Blood pressure	✓	✓	Within 12 hours in acute services Within 1 month of admission to community care Review as part of metabolic monitoring, check BP at each consult
	ECG	✓	✓	Or review recent if available. Request baseline if on psychotropic medications, request routinely annually or more if indicated
	Biochemical assessment			
	FGP – fasting plasma glucose or	✓	Community services collaborate with GP	Within 12 hours in acute services
	RPG – random plasma glucose x 2	✓		Within 1 month of admission to community care Review 3 - 6 monthly for people on antipsychotics and 6 monthly for >65 yr or Aboriginal people >30 yr Perform this AND/OR HbA1c
	HbA1c – haemoglobin A1c			Complete if a person has confirmed diabetes and testing not completed within the last 3 months. If a person does not have confirmed diabetes diagnosis, complete once year only. Request annually, 3 monthly if confirmed T2DM
	Total cholesterol	✓		Within 12 hours in acute services
	LDL – low density lipoprotein	✓		Within 1 month of admission to community care
	HDL – high density lipoprotein	✓		
	Non-HDL cholesterol	✓		Review 6 monthly for people on antipsychotics or >65 yr or Aboriginal people >30 yr Request every 6-12 months, every 3 months if initiating cholesterol medication to monitor progress
	Triglycerides	✓		
	Liver function tests	✓		At least annually, more as indicated
	Urea, electrolytes and creatinine levels	✓		At least annually, more as indicated
	Full blood examination	✓		At least annually, more as indicated
	Estimated GFR	✓		At least annually, more as indicated
	Thyroid function test	✓		Monitor if indicated or on lithium, at least annually
	Prolactin level	✓	✓	Baseline, then at least annually, more as indicated for patients on antipsychotics
	Complete medication history (Polypharmacy, alternative medicines)	✓	✓	Within 12 hours in acute services Within 1 month of admission to community care
	Relevant family medical history	✓	✓	Within 12 hours in acute services Within 1 month of admission to community care
	Substance use	✓	✓	Within 12 hours in acute services Within 1 month admission to community care

Blood borne viruses (Hep C, HIV serology, Hep B)	✓	Community services collaborate with GP	Within 12 hours in acute services Within 1 month of admission to community care Review 3 monthly if regular unprotected sexual contact or intravenous drug use
Oral Health	✓	✓	Within 12 hours in acute services Within 1 month of admission to community care Screen and liaise with dental services if poor, educate consumers on importance of oral hygiene (brushing)
Sleep (disturbance, obstructive sleep apnoea and hypopnea)	✓	✓	12 monthly for people on antipsychotics and for people with BMI>25kg/m ² ESS/STOPBANG/OSA50, measure neck circumference to guide, follow up with GP for sleep study referral if high risk
Falls	✓	✓	Within 2 hours admission to ward Within 1 month of admission to community care

This table outlines minimum expectations. Where possible, monitoring should occur at baseline (prior to medication initiation). Frequency of review is dependent on abnormalities identified and/or changes to medication. Clinicians should consider pathology currency to avoid unnecessary testing. Adapted from: *NSW Health Physical Health Care for People Living with Mental Health Issues 2021 and the RANZCP expert consensus statement for the treatment, management and monitoring of the physical health of people with an enduring psychotic illness 2017.*

Appendix C: Follow-up assessment checklist

Follow-up assessment checklist	
Direct clinical measurements	
☐ Eye checks	☐ Weight
☐ Waist circumference	☐ Blood pressure
☐ Height	☐ Annual ECG
Biochemical assessment	
☐ Fasting blood sugar levels	☐ Low-density lipoprotein (LDL-C)
☐ Two random blood sugar levels (if fasting not available)	☐ High-density lipoprotein (HDL-C)
☐ TG/HDL-C or Log (TG/HDL-C)	☐ Non-HDL cholesterol
☐ Total cholesterol	☐ Triglycerides
Lifestyle factors	
☐ Smoking history	☐ Diet
☐ Daily activity	☐ Sleep
Sociodemographic factors	
☐ Source of income	☐ Type of accommodation
☐ Living with	

Adapted from RANZCP expert consensus statement for the treatment, management and monitoring of the physical health of people with an enduring psychotic illness 2017.

Appendix D: Metabolic Syndrome Early Intervention

Notes:

- **Scope:** Supports early identification and escalation of cardiometabolic risk in mental health consumers; targets and thresholds may vary by guideline or individual context (age, comorbidity, frailty, pregnancy, race, existing CVD)
- **Use current guidance:** Where thresholds/targets differ, follow current best-practice guidance (e.g., Australian Therapeutic Guidelines (eTG), Maudsley, UpToDate) and relevant Australian cardiovascular/diabetes guidance.
- **Restart / step-up monitoring:** Restart (or step-up) monitoring to early-phase timepoints when antipsychotic therapy is initiated or materially changed, or when cardiometabolic risk materially increases (see triggers below).
- **Shared care:** Coordinate management and follow-up with the consumer's GP/specialists; document responsibility for actions and review.

Parameters	Metabolic Risk Parameters	Monitoring Frequency	Interventions	Target Parameters
Family history	Family history of type 2 diabetes, premature CVD, hypertension, dyslipidaemia, obesity , sudden cardiac death/arrhythmia syndromes.	Baseline (update if new information).	Document and incorporate into cardiometabolic risk profile; ensure GP is informed where relevant.	N/A
Lifestyle	Physical inactivity, high sedentary time, poor diet quality, hazardous alcohol use, food insecurity/malnutrition.	Baseline and when clinically indicated.	Lifestyle brief intervention and referral (dietitian, exercise physiology, peer supports, structured programs). Aim for 150–300 min/week moderate OR 75–150 min/week vigorous activity plus strength training at least 2 days/week ; reduce and break up sedentary time. Alcohol: screen and provide brief intervention or referral per national guidance. Diet: encourage health eating habits, consider referral to a dietitian for education	Improved diet quality; increased activity; reduced alcohol-related harm (targets individualised).
Smoking status	Current tobacco smoking (consider vaping/nicotine dependence if relevant).	Baseline and when clinically indicated.	Ask/Advise/Help: brief intervention, motivational support, referral to Quitline/cessation supports. Consider evidence-based pharmacotherapy per current guidelines (eTG/RACGP).	Smoking cessation / reduced nicotine dependence.
Lipids	Use triglycerides (TG) and HDL as MetS markers: TG: ≥ 1.7 mmol/L. HDL: Men < 1.0 mmol/L, Women < 1.3 mmol/L. Note: Total cholesterol is not a MetS criterion; use lipid profile for overall CVD risk assessment.	Baseline, 12 weeks , then every 6 to 12 months	Calculate absolute CVD risk and manage dyslipidaemia with pharmacological and lifestyle interventions as per current Australian CVD risk guidance and best-practice resources (eTG/Maudsley/UpToDate). Coordinate with GP ; refer if severe dyslipidaemia or suspected familial hypercholesterolaemia.	Targets should be risk-based (absolute risk / established CVD), not a single fixed TC/LDL goal for everyone.

Parameters	Metabolic Risk Parameters	Monitoring Frequency	Interventions	Target Parameters
Blood glucose	For MetS marker: fasting plasma glucose ≥ 5.6 mmol/L or known diabetes. If unable to obtain fasting plasma glucose, HbA1c may be used as a screening tool. Use HbA1c for screening/diagnosis per diabetes guidance (diagnosis typically HbA1c $\geq 6.5\%$ on repeat, or fasting glucose ≥ 7.0 mmol/L).	Baseline, 12 weeks, then annually if normal. If prediabetes/diabetes, monitor per diabetes guidance and GP/endocrine plan.	Confirm abnormal results (repeat testing as required), classify (normoglycaemia/prediabetes/diabetes), lifestyle intervention and referral to GP (and endocrinology when indicated). Manage per current diabetes or people at risk of developing type 2 diabetes (prediabetes) guidance (eTG/RACGP/UpToDate).	Glycaemic targets should be individualised (age, comorbidity, hypoglycaemia risk). If diabetes: follow diabetes guideline targets. If no diabetes: aim for normal range.
Blood pressure	MetS marker: ≥ 130mmHg systolic and/or ≥ 85mmHg diastolic (confirm with repeat measures; consider home/ambulatory monitoring).	Baseline, 12 weeks, then at least every 3 months. (earlier if elevated).	Lifestyle intervention; confirm diagnosis; assess absolute CVD risk . Manage hypertension per current guidance (eTG and Australian CVD risk guidance) in collaboration with GP.	Targets should be individualised; commonly $< 140/90$ mmHg for uncomplicated hypertension, lower targets for high-risk consumers or those with comorbidities where tolerated (refer to guidelines).
Obesity / central obesity	BMI: ≥ 25 kg/m² (or ethnicity-adjusted risk) Waist circumference: Men: ≥ 94 cm, Women: ≥ 80 cm (ethnicity-specific cut-offs apply).	Weight/BMI: baseline, 4, 8, 12 weeks, then 3 monthly. Waist: baseline then annually (or more frequent if rapid change).	Structured weight-management supports (dietitian, exercise physiology, behaviour change programs). Review psychotropic regimen and cardiometabolic risk (consider lower-risk alternatives where clinically appropriate). Consider adjunct obesity therapies and/or referral for specialist weight management or metabolic/bariatric assessment in line with current best-practice guidance (e.g. Australian Obesity Management Algorithm , RACGP guidance, Maudsley, UpToDate)	Targets should be realistic and staged (e.g., prevent further gain; 5–10% weight loss where feasible). Waist/BMI targets individualised.
Prolactin	Symptoms of hyperprolactinaemia and/or prolactin above local laboratory reference range (consider macroprolactin and secondary causes where relevant).	Baseline only if clinically indicated (symptoms); repeat if symptomatic or if markedly elevated. For patients on antipsychotics baseline, then annually or as required.	Assess symptoms; exclude pregnancy/thyroid disease and other causes; consider medication review and endocrine referral when appropriate; manage per Maudsley/UpToDate/local endocrine guidance.	Use local laboratory reference ranges.

Triggers to restart/step-up monitoring:

- **Antipsychotic regimen initiated or materially changed** (e.g. start, switch/cross-taper, restart after a break, clinically significant dose increase, or addition of a second antipsychotic / polypharmacy, including changes that increase metabolic risk).
- **New abnormal metabolic finding or clinically significant deterioration** (e.g. new prediabetes/diabetes, new hypertension, marked dyslipidaemia, rapid weight gain or increasing waist circumference).
- **New major risk factor or change in clinical context** (e.g. pregnancy/postpartum, initiation of medicines that increase metabolic risk such as systemic corticosteroids, major lifestyle change, or new diagnosis that increases baseline CVD risk).

This document can be made available in alternative formats on request.

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